

SENATE BILL 1767

By Bowling

AN ACT to amend Tennessee Code Annotated, Title 33;  
Title 37; Title 44; Title 47; Title 49; Title 53; Title 63  
and Title 68, relative to health.

WHEREAS, substantial evidence reveals that mRNA pharmaceutical interventions are ineffective at preventing infection, transmission, severe illness, hospitalization, or death, with additional concerns over rapidly waning immunity and heightened risks associated with their use; and

WHEREAS, substantial evidence indicates that mRNA pharmaceutical interventions are associated with widespread and severe adverse events, including thousands of documented injuries, underreported cases, and significant increases in pharmaceutical-related harm and excess mortality; and

WHEREAS, numerous health organizations, medical professionals, and international governments have called for an immediate moratorium on mRNA pharmaceutical interventions, citing serious health concerns, documented risks, and the urgent need for comprehensive investigations into their safety and efficacy; and

WHEREAS, mRNA pharmaceutical interventions exhibit wide-area biodistribution and bioaccumulation, with spike proteins and mRNA sequences detected in critical organs such as the brain, heart, and endocrine glands, as well as in the blood, placenta, and fetus, raising serious concerns about systemic and long-term effects on human health; and

WHEREAS, components of mRNA vaccines, including spike proteins and mRNA sequences, have been shown to persist in the body far longer than initially claimed, up to 245 days in some cases, raising critical concerns about their long-term effects on human health and systemic stability; and

WHEREAS, mRNA pharmaceutical interventions have been strongly associated with severe cardiovascular and thrombotic events, including myocarditis, pericarditis, blood clots, and cardiomyopathy, with documented increases in adverse event reports and sudden deaths, particularly among young individuals, raising urgent concerns about their safety and widespread use; and

WHEREAS, mRNA pharmaceutical interventions have been linked to severe lung and respiratory complications, including increased risks of asthma, acute respiratory distress syndrome, pulmonary fibrosis exacerbation, and systemic respiratory disorders, highlighting significant concerns about their impact on respiratory health; and

WHEREAS, mRNA pharmaceutical interventions have been implicated in endocrine disruptions, including the occurrence of abnormal blood glucose levels, raising concerns about their effects on metabolic and hormonal health; and

WHEREAS, mRNA pharmaceutical interventions have been associated with significant pediatric risks, including increased disease prevalence in infants, heightened rates of heart disease, anxiety, depression, and adverse neurological effects in children, as well as alarming impacts on offspring development, raising profound concerns about their safety in vulnerable populations; and

WHEREAS, mRNA pharmaceutical interventions have been associated with cancer-related risks, including the presence of spike proteins that may support cancer cell survival, contamination with SV40 sequences linked to cancer, stimulation of tumor growth, and significant increases in cancer diagnoses and deaths following mass vaccination campaigns, raising grave concerns about their safety and long-term effects; and

WHEREAS, mRNA pharmaceutical interventions have been linked to a range of severe neurological effects, including Guillain-Barré syndrome, seizures, autoimmune brain inflammation, neuromuscular diseases, and increased risks of neurological disorders such as

Bell's palsy and encephalitis, raising critical concerns about their impact on the nervous system and overall brain health; and

WHEREAS, mRNA pharmaceutical interventions have been associated with lymphatic system disorders, including vaccine-induced lymphadenopathy and axillary lymph node abnormalities, raising concerns about their effects on immune system function and lymphatic health; and

WHEREAS, mRNA pharmaceutical interventions have been linked to an increase in psychological disorders, including anxiety, depression, and new-onset psychosis, exacerbating mental health concerns; and

WHEREAS, mRNA pharmaceutical interventions have been associated with the onset of diabetes-related conditions, including new-onset Type 1 and Type 2 diabetes, transient hyperglycemia, and central diabetes insipidus, raising concerns about their impact on metabolic health and glycemic regulation; and

WHEREAS, mRNA pharmaceutical interventions have been linked to a concerning incidence of sleep disorders, raising additional health concerns about their potential neurological and systemic side effects; and

WHEREAS, mRNA pharmaceutical interventions have been associated with nerve-related complications, including radial nerve motor palsy and trigeminal neuralgia, raising concerns about their effects on the peripheral nervous system; and

WHEREAS, mRNA pharmaceutical interventions have been linked to a wide range of dermatological and skin-related adverse events, including new-onset and exacerbation of psoriasis, vitiligo, severe dermatitis, alopecia, toxic epidermal necrolysis, and other inflammatory and autoimmune skin conditions, raising serious concerns about their impact on dermatological health; and

WHEREAS, mRNA pharmaceutical interventions have been associated with serious ocular complications, including eye inflammation, blood supply deficiencies, blindness, white dot syndrome, and Henoch-Schönlein purpura, raising concerns about their safety and impact on vision and ocular health; and

WHEREAS, mRNA pharmaceutical interventions have been linked to significant reproductive and fertility concerns, including menstrual irregularities, higher rates of miscarriage, functional lactation disorders, fetal exposure risks, and negative impacts on sperm quality, raising substantial concerns about their effects on reproductive health and outcomes; and

WHEREAS, mRNA pharmaceutical interventions have been associated with a broad range of immune system and autoimmune disorders, including reduced immune function, lupus, rheumatoid arthritis, autoimmune encephalitis, and the onset or exacerbation of various autoimmune diseases, raising critical concerns about their impact on immune regulation and systemic health; and

WHEREAS, self-amplifying mRNA (samRNA) vaccines pose significant risks, including the potential for injecting live, self-replicating viruses into recipients due to contamination, and the production of even more toxic spike proteins in the body compared to regular mRNA vaccines, raising serious concerns about their safety and regulatory oversight; and

WHEREAS, there have been no long-term, double-blind, placebo-controlled, randomized studies to assess the safety of mRNA and self-amplifying mRNA (samRNA) pharmaceutical interventions in agricultural produce, despite mounting evidence of severe adverse effects in humans, animals, and the environment, including immune system disruptions, cardiovascular risks such as SD40 and DNA integration, raising significant concerns about the unknown and potentially harmful impact of these technologies on food safety, animal welfare, and human health; and

WHEREAS, readily available, safe, and effective FDA-approved treatments, such as budesonide, ivermectin, hydroxychloroquine, and monoclonal antibodies, have demonstrated significant efficacy in the early and late treatment of COVID-19, with expert testimony and clinical studies confirming their potential to prevent severe outcomes and reduce mortality, raising concerns about the prioritization of experimental vaccines over these proven therapies; now, therefore,

BE IT ENACTED BY THE GENERAL ASSEMBLY OF THE STATE OF TENNESSEE:

SECTION 1. This act is known and may be cited as the "Tennessee mRNA Pharmaceutical Sovereignty and Safety Act."

SECTION 2. Tennessee Code Annotated, Title 63, Chapter 1, Part 1, is amended by adding the following as a new section:

(a) As used in this section:

(1) "Healthcare provider" means an individual who is licensed, registered, certified, or otherwise permitted under this title or title 68 to engage in the provision of health care or veterinary medicine in the course of a profession; and

(2) "mRNA vaccine or vaccine material":

(A) Means a substance to stimulate the production of antibodies and provide immunity against disease by introducing messenger ribonucleic acid (mRNA) that corresponds to a viral protein; and

(B) Includes a self-amplifying mRNA (samRNA) vaccine or vaccine material.

(b) An individual, including a healthcare provider, shall not administer to any human or animal in this state a vaccine or other injectable solution that contains an mRNA vaccine or vaccine material.

(c)

(1) A violation of subsection (b) is a Class A misdemeanor, punishable only by a fine of up to two thousand five hundred dollars (\$2,500) per violation. The administration of each vaccine or other injectable solution in violation of subsection (b) constitutes a separate violation.

(2) In addition to subdivision (c)(1), a healthcare provider who violates subsection (b) may be subject to administrative sanction or penalty by the healthcare provider's regulatory entity, up to and including revocation of the healthcare provider's license, registration, certification, or other permission to engage in the provision of health care or veterinary medicine in this state.

SECTION 3. This act takes effect January 1, 2027, the public welfare requiring it.